



TEAM CLINICALS

DYNAMIC DIAGNOSERS



CASE PERFORMANCE

CORRELATING THE ACADEMIC KNOWLEDGE
INTO CLINICALS

PREFACE

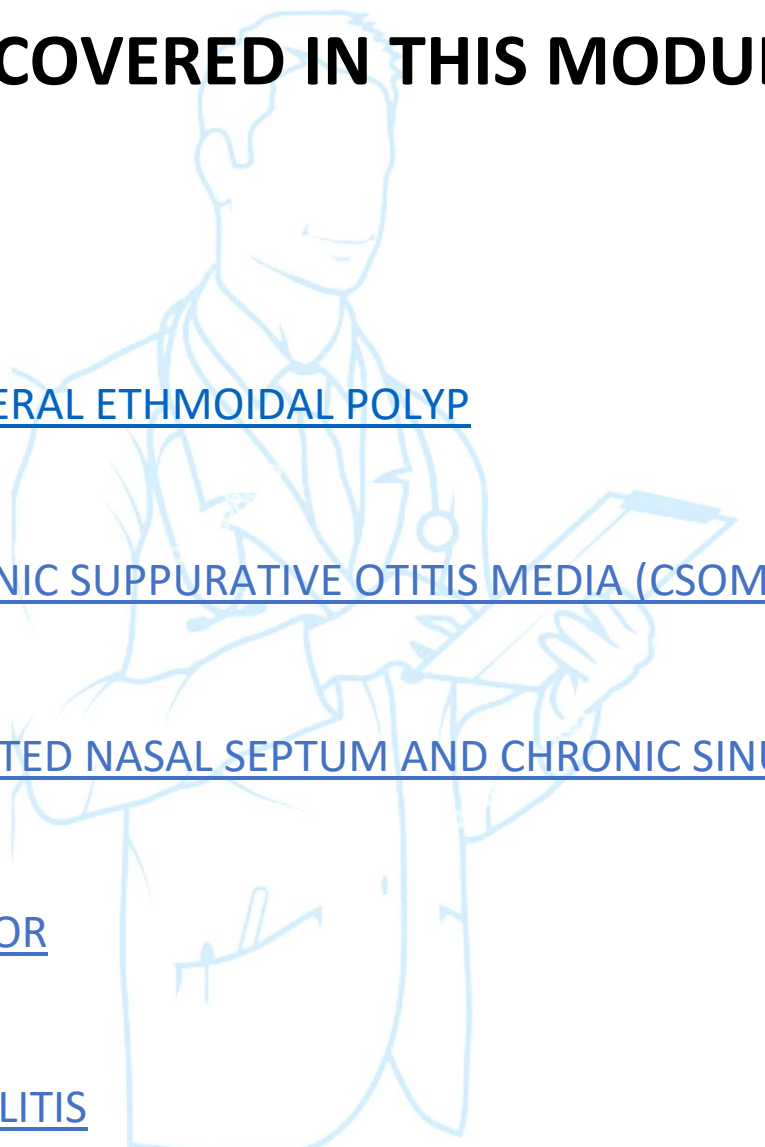
Greetings from Dr. Nerve.....!!!

This is Dynamic Diagnosticians from Dr. Nerve. We have launched this team to acknowledge you about case presentation which includes history taking and examination. The viewpoint of this project is to make it useful for your clinical studies as reference. The following are the departments covered by us,

- ❖ ENT
- ❖ ORTHOPAEDICS
- ❖ PEDIATRICS
- ❖ COMMUNITY MEDICINE
- ❖ OBSTETRICS AND GYNAECOLOGY
- ❖ GENERAL MEDICINE
- ❖ GENERAL SURGERY

The case Performa for ENT has been uploaded here. Kindly make use of it.....

CASES COVERED IN THIS MODULE.....

- 
- ❖ BILATERAL ETHMOIDAL POLYP
 - ❖ CHRONIC SUPPURATIVE OTITIS MEDIA (CSOM)
 - ❖ DEVIATED NASAL SEPTUM AND CHRONIC SINUSITIS
 - ❖ STRIDOR
 - ❖ TONSILITIS

BILATERAL ETHMOIDAL POLYP

Name

Sex

Age

Occupation

Address

CHIEF COMPLAINTS

- Nasal obstruction and bleeding for ____ days
- Headache and loss of smell sensation for ____ days

HISTORY OF PRESENT ILLNESS

The patient was apparently normal before ____ days

NASAL OBSTRUCTION

- Onset: insidious or sudden
- Progressive or not
- Duration
- Aggravating factor
- Relieving factor

NASAL DISCHARGE

- Side
- Duration
- Quantity
- Foul-smelling
- Bloodstained

EPISTAXIS

- Side
- Duration
- Quantity
- Anterior or posterior
- History of trauma or nose pricking
- Frequency

NASAL DISCHARGE

- Side
- Duration
- Quantity
- Foul smelling
- Blood stained

FACIAL PAIN

- Site
- Duration
- Aggravating factor
- Relieving factor

LOSS OF SMELL

CHANGE IN VOICE

H/O Running nose, itching, sneezing and watery eyes

H/O head ache

H/O epistaxis

H/O fever

H/O throat pain ear ache/decreased hearing/ear discharge

H/O snoring/ mouth breathing

PAST HISTORY

- H/O similar complaint
- H/O diabetes, hypertension, asthma, TB
- H/O previous surgery
- H/O drug allergy

PERSONAL HISTORY

- Diet
- Alteration in appetite
- Sleep disturbance
- Bowel and bladder movements
- Smoking, alcohol, tobacco

FAMILY HISTORY

- Any significant family history

GENERAL EXAMINATION

Build, conscious, well-built and oriented to place , time, place and person and well nourished

- Pallor
- Icterus
- Cyanosis
- Clubbing
- Pedal oedema
- Lymphadenopathy

VITAL SIGNS

- Temperature
- Pulse rate
- Respiratory rate

- Blood pressure

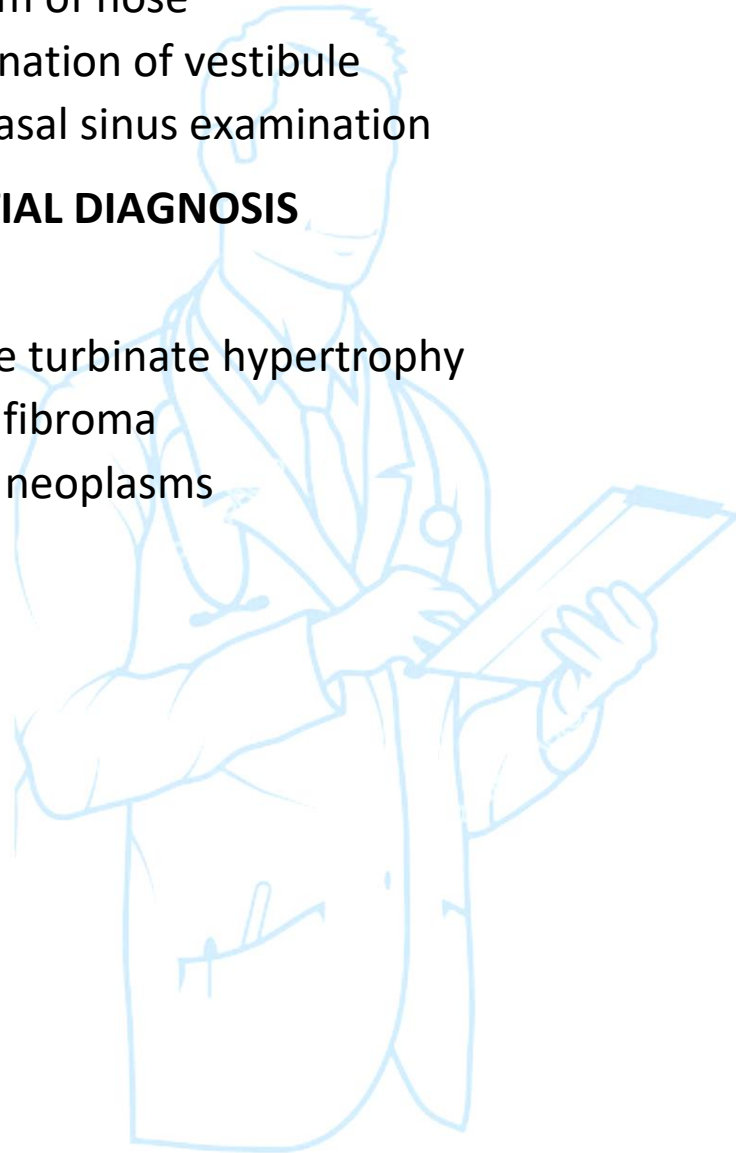
ENT EXAMINATION

NOSE

- External appearance
- Dorsum of nose
- Examination of vestibule
- Paranasal sinus examination

DIFFERENTIAL DIAGNOSIS

- Polyp
- Middle turbinate hypertrophy
- Angio fibroma
- Other neoplasms



CASE SHEET

Name: XXX

Sex: Male

Age: 45

Occupation: IT

Address: Trichy

CHIEF COMPLAINTS

- Nasal obstruction for the past 1 month
- Loss of smell for past 1 month
- Headache for past one week
- Watery nasal discharge for past one week

HISTORY OF PRESENT ILLNESS

Patient was apparently normal before one month

NASAL OBSTRUCTION

- Side – both sides
- Onset : insidious
- Progressive - gradual
- Duration – 1 month
- Aggravating factor - lying
- Relieving factor - medication

NASAL DISCHARGE

- Side – both sides
- Duration - one week
- Quantity - watery
- Foul smelling - no

- Blood stained - no

LOSS OF SMELL – past one week

CHANGE IN VOICE - no

H/O Running nose, sneezing and watery eyes for 1 week

H/O head ache for past 1 week

No H/O epistaxis

No H/O throat pain ear ache/decreased hearing.

H/O snoring/ mouth breathing for past one month

PAST HISTORY

- No H/O similar complaint
- No H/O diabetes, hypertension, asthma, TB
- No H/O previous surgery
- No H/O drug allergy

PERSONEL HISTORY

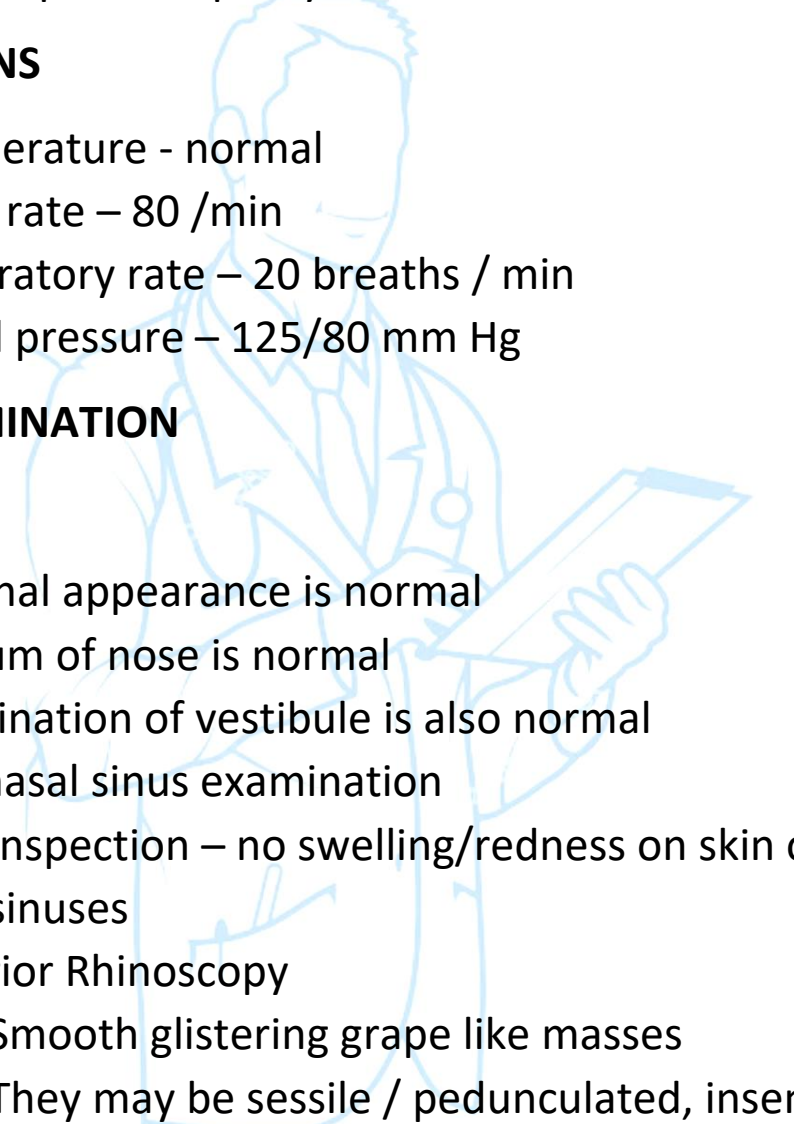
- Diet - mixed
- Alteration in appetite - no
- Sleep disturbance - no
- Bowel and bladder movements - normal
- Smoking, alcohol, tobacco - no

FAMILY HISTORY

No significant family history

GENERAL EXAMINATION

Moderately Build, conscious, well oriented to place ,
time, place and person and well nourished

- 
- No Pallor
 - No Icterus
 - No Cyanosis
 - No Clubbing
 - No Pedal oedema
 - No Lymphadenopathy

VITAL SIGNS

- Temperature - normal
- Pulse rate – 80 /min
- Respiratory rate – 20 breaths / min
- Blood pressure – 125/80 mm Hg

ENT EXAMINATION

NOSE

- External appearance is normal
- Dorsum of nose is normal
- Examination of vestibule is also normal
- Paranasal sinus examination
 - Inspection – no swelling/redness on skin over the sinuses
- Anterior Rhinoscopy
 - Smooth glistening grape like masses
 - They may be sessile / pedunculated, insensitive to probing and do not bleed on touching
 - A polyp may protrude from nostril and appears pink and vascular stimulating neoplasms

PROVISIONAL DIAGNOSIS

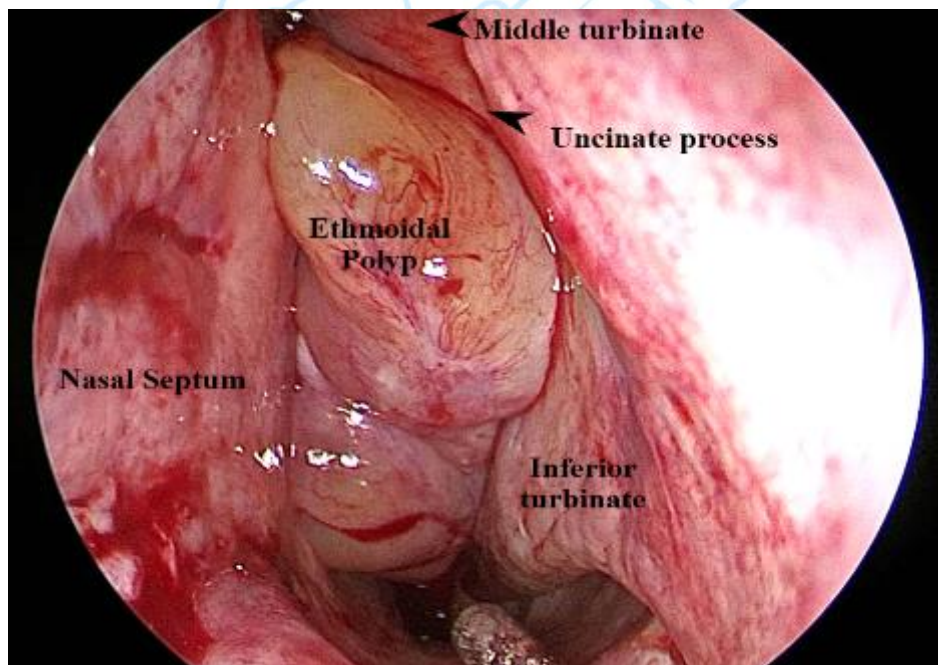
- Bilateral ethmoidal polyp, hypertrophy of turbinate and cystic middle turbinate

INVESTIGATIONS

CT scan of paranasal sinuses

TREATMENT

- Early polypoidal changes with oedematous mucosa may revert to normal with antihistaminic and control of allergy.
- Ethmoidal polyp are removed by endoscopic sinus surgery, more popularly called Functional Endoscopic Sinus Surgery (FESS)



CHRONIC SUPPURATIVE OTITIS MEDIA (CSOM)

Name

Age

Sex

Occupation

Address

Socioeconomic status

CHIEF COMPLAINTS

Complaints in the patient's own words

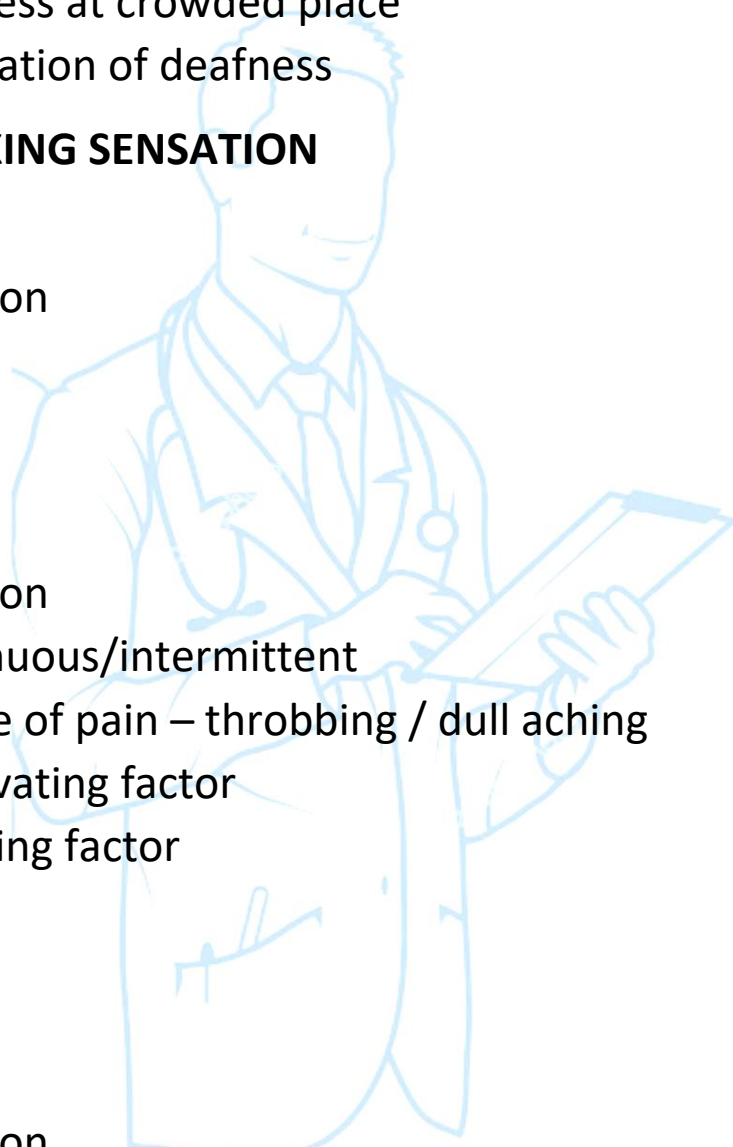
HISTORY OF PRESENT ILLNESS

The patient was apparently normal before _____

EAR DISCHARGE

- Side
- Onset
- Duration
- Amount
- Colour
- Aggravating factor
- Relieving factor
- Foul smelling or not
- Blood-stained or not
- Stage of discharge – active/inactive / quiescent

HARD OF HEARING

- 
- Side
 - Onset
 - Duration
 - Deafness at time of discharge
 - Progression
 - Deafness at crowded place
 - Fluctuation of deafness

EAR BLOCKING SENSATION

- Side
- Duration

EAR PAIN

- Side
- Onset
- Duration
- Continuous/intermittent
- Nature of pain – throbbing / dull aching
- Aggravating factor
- Relieving factor

TINNITUS

- Side
- Onset
- Duration
- Nature of sound
- Subjective or objective

VERTIGO

- Onset
- Duration
- No. of episodes
- Last episode

H/O headache, recent sore throat, common cold

H/O dizziness

H/O diplopia

H/O nausea and vomiting

H/O facial paralysis, pain and swelling

H/O convulsions, neck rigidity

H/O tooth ache

PAST HISTORY

- H/O previous similar episodes
- H/O DM, HT, TB, epilepsy, asthma
- H/O bleeding disorder

PERSONAL HISTORY

- Diet
- Smoking
- Alcoholic

FAMILY HISTORY

- H/O similar complaints
- H/O bleeding disorder
- H/O allergy

MENSTRUAL HISTORY

- Regular cycle
- Age of menarche / menopause

GENERAL EXAMINATION

Build, conscious, well oriented to place , time, place and person and well nourished

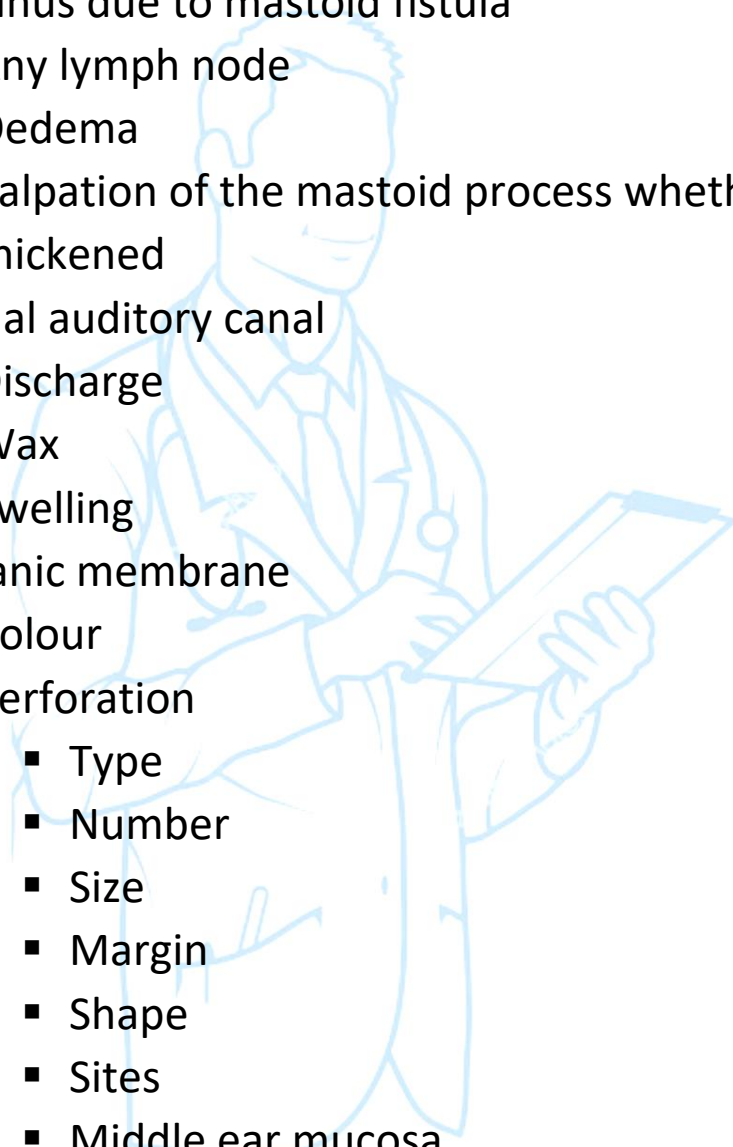
- Pallor
- Icterus
- Cyanosis
- Clubbing
- Pedal oedema
- Lymphadenopathy

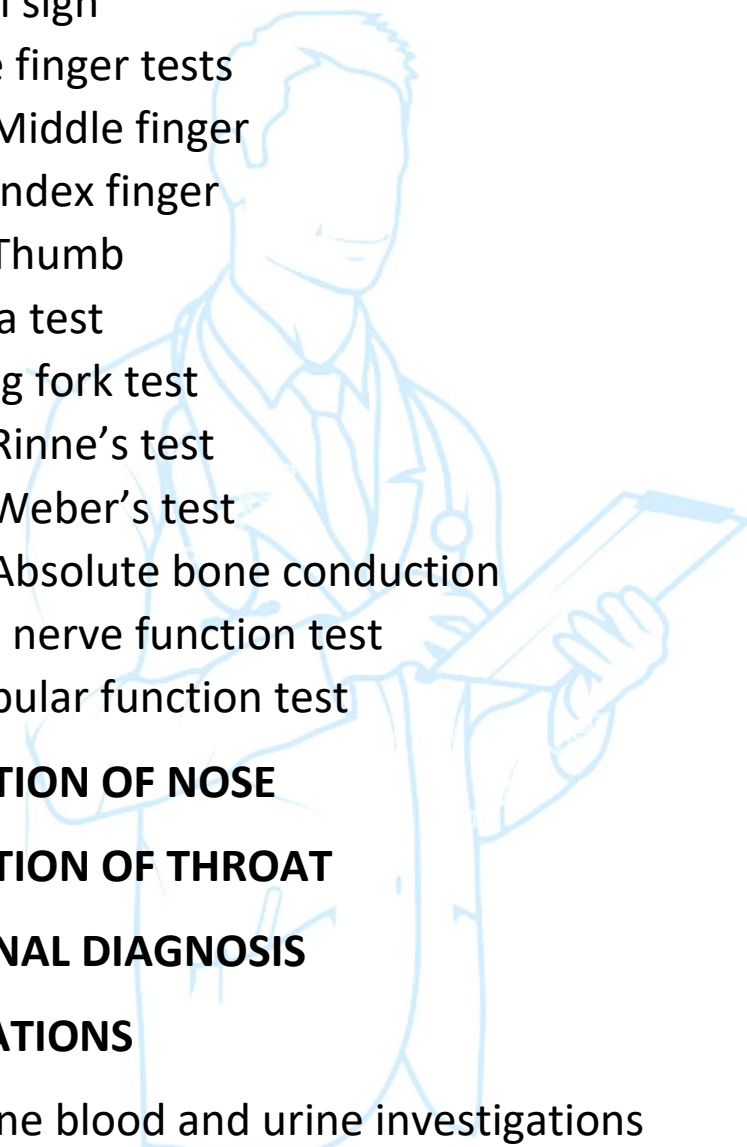
VITAL SIGNS

- Temperature
- Pulse rate
- Respiratory rate
- Blood pressure

EXAMINATION OF EAR

- Preauricular region
 - Preauricular sinus
 - Accessory tragus
- Pinna
 - Size – macrotia / microtia / anotia
 - Congenital anomalies
 - Thickened or cauliflower pinna
 - Normal position

- 
- Post auricular region
 - Post auricular groove – normal/deepened/obliterated
 - Swelling
 - Scar of previous surgery
 - Sinus due to mastoid fistula
 - Any lymph node
 - Oedema
 - Palpation of the mastoid process whether thickened
 - External auditory canal
 - Discharge
 - Wax
 - Swelling
 - Tympanic membrane
 - Colour
 - Perforation
 - Type
 - Number
 - Size
 - Margin
 - Shape
 - Sites
 - Middle ear mucosa
 - Colorization
 - Any discharge through the perforation
 - Handle of malleolus
 - Cone of light
 - Quadrants

- 
- Granulation
 - Retraction pocket
 - Incudostapedial joint
 - Congestion
 - Mobility
 - Tragal sign
 - Three finger tests
 - Middle finger
 - Index finger
 - Thumb
 - Fistula test
 - Tuning fork test
 - Rinne's test
 - Weber's test
 - Absolute bone conduction
 - Facial nerve function test
 - Vestibular function test

EXAMINATION OF NOSE

EXAMINATION OF THROAT

PROVISIONAL DIAGNOSIS

INVESTIGATIONS

Routine blood and urine investigations

X-ray mastoid (Schuller's view both sides)

CSOM --- CASE SHEET

Name - Ram

Age - 20

Sex - male

Occupation - Student

Address - Karur

CHIEF COMPLAINTS

- Discharge from left ear since 2 months but has recently increased since 5 days
- Decreasing hearing from left ear since 4 days

HISTORY OF PRESENT ILLNESS

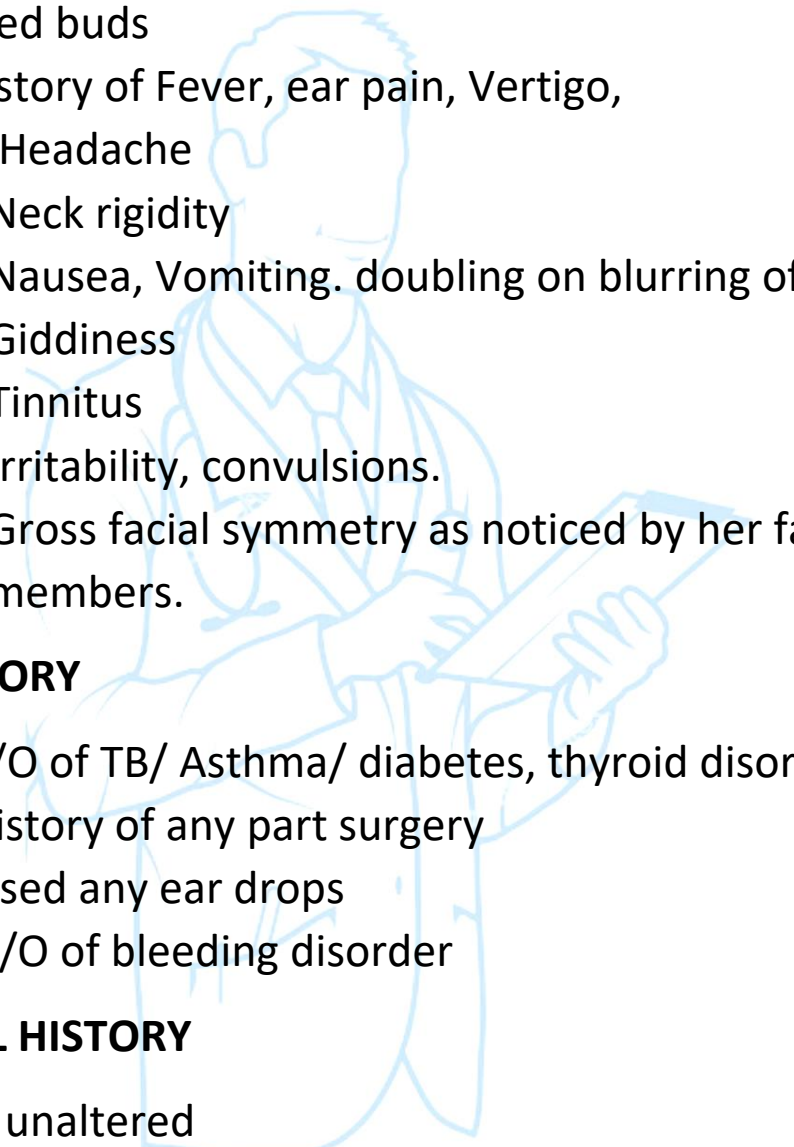
The patient was apparently normal before 2 months

EAR DISCHARGE

- Side – left
- Onset - insidious
- Duration – 2 months
- Nature - intermittent
- Amount - profuse
- Colour – mucopurulent yellowish
- Foul smelling - no
- Blood stained - no

HEARING LOSS

- Side - left
- Onset - insidious
- Duration – 4 days

- 
- H/O= upper respiratory tract infection [URT infection. ↑ discharge]
 - No H/O: recent sore throat, common cold.
 - He used matchstick for cleaning ears, but last 2 months he used buds
 - No history of Fever, ear pain, Vertigo,
 - Headache
 - Neck rigidity
 - Nausea, Vomiting. doubling on blurring of vision.
 - Giddiness
 - Tinnitus
 - Irritability, convulsions.
 - Gross facial symmetry as noticed by her family members.

PAST HISTORY

- No H/O of TB/ Asthma/ diabetes, thyroid disorder.
- No history of any part surgery
- Not used any ear drops
- No H/O of bleeding disorder

PERSONAL HISTORY

- sleep unaltered
- Diet: pure veg
- Denies any addiction
- Bladder and bowel movements unaltered.

FAMILY HISTORY

- No H/o similar complaints in family

- No bleeding disorder
- No history of allergy

Summary

- A 20 year old boy who presented with chronic ear discharge and hearing loss is probable suffering from mucosal type of CSOM, which is currently active, most probably due to trauma with matchstick and car buds.

GENERAL EXAMINATION

Build, conscious, well oriented to place, time, place and person and well nourished

- Pallor
- Icterus
- Cyanosis
- Clubbing
- Pedal oedema
- Lymphadenopathy

VITAL SIGNS

- Temperature
- Pulse rate
- Respiratory rate
- Blood pressure

ENT EXAMINATION

	Right ear	Left ear
Preauricular area	Normal	Normal
Pinna	Normal	Normal

Postauricular area	Normal	Normal
Tragal sign	Negative	Normal
External Auditory Canal (seen in Bulls eye lamps)	Clear	Clear
Tympanic membrane (seen using Bulls eye lamp)	normal	central perforation

TYMPANIC MEMBRANE

RIGHT EAR	LEFT EAR
• Normal	• There is central perforation majorly in anterioposterior quadrant of size 3*3 mm surrounded by annulus on all sides.
• Pearly white in colour	• Pars tensa appears dull
• Cone of light is appreciated in anterioinferior quadrant	• Handle of malleus appear slightly foreshortened
• Anterior and posterior malleolar fold are normal	• Anterior and posterior fold are visible
• Rest of tympanic membrane is normal	• Rest of tympanic membrane is congested
• Pars flaccid is normal	• Pars flaccid is normal

MIDDLE EAR

• Not seen since tympanic membrane is intact	• Could not be appreciated
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MASTOID

• Normal	• Normal
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FACIAL NERVE

• Normal	• Normal
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TUNNING FORK FOR TESTS

RINNE's TEST

+ve	-ve
• 256 Hz +	• 256 Hz -
• 512 Hz +	• 512 Hz -
• 1024 Hz +	• 1024 Hz +

ABSOLUT BONE CONDUCTION

• Same as examiner	• Same as examiner
--------------------	--------------------

Nose examination

- vestibule is normal
- Nasal septum appears normal.
- sinus tenderness is absent
- Trachea appears normal

Oral cavity examination

- Lips normal
- Gums & Teeth = normal
- Tongue – normal
- Anterior tonsillar pillar = normal

- Tonsils not seen.
- Posterior tonsillar pillar is not appreciated
- Posterior pharyngeal wall - normal

Neck examination

- Trials sign is negative
- No swelling can be appreciated.
- Trachea is in centre on palpation
- B/L carotid arteries are palpable.
- Rest of neck appears normal.

Provisional diagnosis

- This is a case of left sided active mucosal CSOM, due to trauma by matchsticks and ear buds along with moderate conductive hearing loss on same side and without any extracranial (on) intracranial complications.



DEVIATED NASAL SEPTUM AND CHRONIC SINUSITIS

Name:

Age:

Sex:

Religion:

Occupation:

Address:

CHIEF COMPLAINTS:

- H/O Nasal obstruction - _____ days
- H/O Head ache- _____ days
- H/O Loss of sensation and smell- _____ days

History of present illness:-

- The patient was apparently normal for past _____ days.
Then he/she develops,

NASAL OBSTRUCTION

- Onset : insidious or sudden
- Progressive or not
- Duration :
- Aggravating factor :-
- Relieving factor :-

HEAD ACHE :-

- Onset :-
- Progressive or not
- Duration
- Continuous / intermittent :-

- Aggravating factor :-
- Relieving factor :-
- Associated with nausea, vomiting, blurring of vision

LOSS OF SENSATION AND SMELL :-

- Onset :- Gradual/Insidious
- Duration
- Nature :- Progressive or not

NEGATIVE HISTORY :-

- H/O past nasal discharge
- H/O nasal regurgitation
- H/O voice change
- H/O external nose deformity
- H/O sneezing
- H/O mouth breathing
- H/O epistaxis
- H/O difficulty in breathing
- H/O difficulty in swallowing
- H/O recurrent cough
- H/O blurring of vision
- H/O ear ache
- H/O tinnitus

PAST HISTORY :-

- H/O any previous similar complaints
- H/O diabetes mellitus
- H/O tuberculosis
- H/O hypertension
- H/O allergy, asthma

- H/O previous surgery
- H/O blood transfusion

PERSONAL HISTORY :-

- DIET :- Veg or mixed
- Smoking :-
- Alcoholic :-
- Marital status :-
- Degree of consanguinity :-
- Menstrual history :-

FAMILY HISTORY :-

- H/O similar complaints in the family
- H/O significant disease

TREATMENT HISTORY :-

- H/O Previous treatment

EXAMINATION

GENERAL EXAMINATION

- Well oriented to place / time / person
- Well / moderately built and nourished
- Pallor
- Icterus
- Cyanosis
- Clubbing
- Edema

- Lymphadenopathy

VITALS

- TEMPERATURE
- PULSE
- BLOOD PRESSURE
- RESPIRATORY RATE

ENT EXAMINATIONS

EXTERNAL APPERANCE :-

- Any deformity
- Nasal bridge
- Any scar or sinus

ANTERIOR RHINOSCOPIC EXAMINATION:-

- Appearance of vestibule
- Deviation of nasal septum
- Turbinate hypertrophy
- nasal mucosa – congested or normal

POSTERIOR RHINOSCOPIC EXAMINATION:-

- Secretions – present/not
- Adenoids
- Area around eustachian tube – congested or normal

FUNCTIONAL EXAMINATION :-

- Cotton wool test
- Cold spatula test
- Cottles test

ORAL CAVITY AND OROPHARYNX:-

- Swelling
- Congestion
- Secretion

EXAMINATION OF EYE :-

- Phobia
- Field of vision
- Movement of eyeball
- Corneal reflex

EXAMINATION OF EAR :-

- Hearing loss
- Discharge
- Pain
- Tinnitus

EXAMINATION CVS :-

- Cardia sounds
- Any murmur

EXAMINATION OF RS :-

- Vesicular breath sounds

EXAMINATION OF SINUSES :-

- Any tenderness on maxillary, ethmoid, and frontal sinus

CASE SHEET

Name: Vinoth

Age: 21

Sex: Male

Religion: Hindu

Occupation: Student

Address: Musiri

CHIEF COMPLAINTS:

- H/O Nasal obstruction for 3 months
- H/O Head ache for 3 months
- H/O Loss of sensation and smell for 3 months

History of present illness:-

- The patient was apparently normal for past 3 months. Then he develops,

NASAL OBSTRUCTION

- Onset : insidious
- Progressive in nature
- Duration : 3 months
- Aggravated by exposure to cold
- Relieving factor :- No

HEAD ACHE :-

- Site – area between two eyes
- Duration :- 3 months
- Intermittent in nature
- Aggravating factor :- exposure to cold

- Relieving factor :- medications
- Not Associated with nausea, vomiting, blurring of vision

LOSS OF SENSATION AND SMELL :-

- Onset :- Sudden
- Duration :- 3 months
- Nature :- Progressive

NEGATIVE HISTORY :-

- No H/O past nasal discharge
- No H/O nasal regurgitation
- No H/O voice change
- **H/O external nose deformity**
- No H/O sneezing
- **H/O mouth breathing**
- No H/O epistaxis
- **H/O difficulty in breathing**
- No H/O difficulty in swallowing
- No H/O recurrent cough
- No H/O blurring of vision
- No H/O ear ache
- No H/O tinnitus

PAST HISTORY :-

- No H/O any previous similar complaints
- No H/O diabetes mellitus
- No H/O tuberculosis
- No H/O hypertension
- No H/O allergy, asthma
- No H/O previous surgery

- No H/O blood transfusion

PERSONAL HISTORY :-

- DIET :- Mixed
- Smoking :- No
- Alcoholic :- No
- Marital status :- Single

FAMILY HISTORY :-

- No H/O similar complaints in the family
- No H/O significant disease

TREATMENT HISTORY :-

- No H/O Previous treatment

EXAMINATION

GENERAL EXAMINATION

- Well oriented to place / time / person
- Moderately built and nourished
- No Pallor
- No Icterus
- No Cyanosis
- No Clubbing
- No Oedema
- No Lymphadenopathy

VITALS

- TEMPERATURE :- 100 F

- PULSE :-76 beats/min
- BLOOD PRESSURE :- 120/80 mm Hg
- RESPIRATORY RATE :-15 breaths/min

ENT EXAMINATIONS

EXTERNAL APPERANCE :-

- Depression on the skin
- Nasal bridge is normal
- No scar or sinus

ANTERIOR RHINOSCOPIC EXAMINATION:-

- Appearance of vestibule is normal
- Deviation of nasal septum on left sides
- Turbinate hypertrophy – right inferior
- nasal mucosa – congested

POSTERIOR RHINOSCOPIC EXAMINATION:-

- Secretions – No
- Adenoids - No
- No abnormalities seen in eustachian tube opening areas

FUNCTIONAL EXAMINATION :-

- Cotton wool test – positive for right side
- Cold spatula test – mist formation less in left side
- Cottles test – positive for left side

ORAL CAVITY AND OROPHARYNX:-

- Normal

EXAMINATION OF EYE :-

- Normal

EXAMINATION OF EAR :-

- Normal

EXAMINATION CVS :-

- Cardia sounds – S1 and S2 is heard
- Any murmur - No

EXAMINATION OF RS :-

- Normal Vesicular breath sounds heard

EXAMINATION OF SINUSES :-

- Maxillary sinus is normal
- Ethmoidal sinus – tenderness is present
- Frontal sinus – normal

INVESTIGATIONS :-

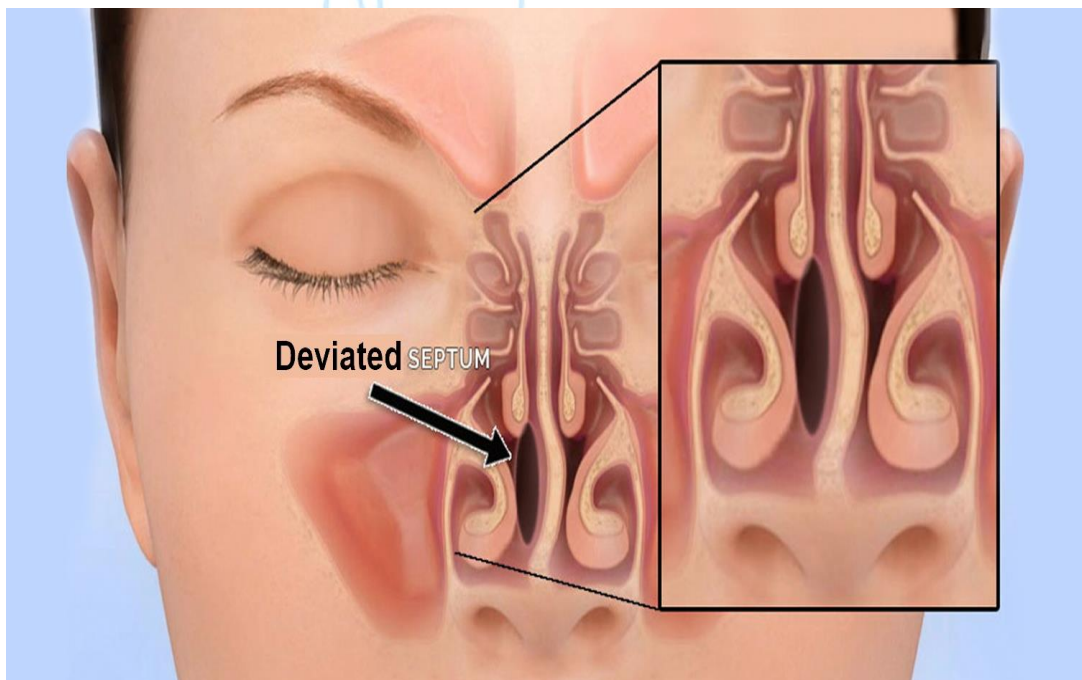
- X-ray (Waters view)
- CT SCAN
- CBC
- Blood grouping and typing
- Diagnostic nasal endoscopy

DIAGNOSIS :-

- Deviation of nasal septum to left side
- Right inferior turbinate hypertrophy
- Chronic sinusitis involved in ethmoidal air sinus

TREATMENT :-

- For deviated nasal septum---submucosal resection
- For sinusitis--- medical treatment with antibiotics
- If not cured---FESS(Functional Endoscopic Sinus Surgery)



STRIDOR

Name:

Age:

Sex:

Religion:

Occupation:

Address:

CHIEF COMPLAINTS:

- H/O Cough - _____ days
- H/O Breathlessness- _____ days
- H/O sounds during breathing _____ days

History of present illness:-

- The patient was apparently normal for past _____ days.
Then he/she develops,

COUGH:-

- Onset : insidious or sudden
- Progressive or not
- Duration :
- Paroxysmal / persistence
- Dry / productive
- Postural variation
- Diurnal variation
- Aggravating factor
- Relieving factor :-
- Associated with chest pain / syncope / wheezing

BREATHLESSNESS :-

- Onset
- Progressive or not
- Duration
- Grade
- Postural variation
- Diurnal variation
- PND / Orthopnoea
- Aggravating factor
- Relieving factor

SOUNDS DURING BREATHING :-

- Onset
- Duration
- Aggravating factor
- Relieving factor

PAST HISTORY :-

- H/O any previous similar complaints
- H/O diabetes mellitus
- H/O tuberculosis
- H/O hypertension
- H/O allergy, asthma
- H/O previous surgery
- H/O pleural effusion
- H/O pneumonia
- H/O past respiratory infection

FAMILY HISTORY :-

- H/O similar complaints in the family
- H/O other systemic disease in the family

TREATMENT HISTORY :-

- H/O previous treatment

NEGATIVE HISTORY :-

- H/O seizures
- H/O fever
- H/O sleeplessness
- H/O tetany

PERSONAL HISTORY :-

- DIET :- Veg or mixed
- Smoking
- Alcoholic
- Marital status
- Degree of consanguinity
- Exposure to any occupational disorder

EXAMINATION

VITALS

- TEMPERATURE
- PULSE
- BLOOD PRESSURE
- RESPIRATORY RATE

GENERAL EXAMINATION

- Well oriented to place / time / person
- Well / moderately built and nourished
- Pallor
- Icterus
- Cyanosis
- Clubbing
- Oedema
- Lymphadenopathy

INSPECTION :-

- Nasal septum
- Polyp
- Chest
 - ❖ Symmetry
 - ❖ Shape
 - ❖ Moves equally with respiratory movement
 - ❖ Indrawing / Intercostal fullness
 - ❖ Scar
- Tracheal position
- Apical impulse

PALPATION :-

- Tracheal position
- Apical impulse
- Chest movement
- Chest expansion measurements
- Tactile fremitus

PERCUSSION :- (Lungs)

- Supraclavicular
- Infraclavicular
- Mammary
- Inframammary
- Axillary
- Infra axillary
- Suprascapular
- Infra scapular
- Interscapular

AUSCULTATION :-

- Normal breath sounds
- Bronchial sounds
- Added sounds
- Crepitation
- Ronchi
- Pleural rub
- Vocal resonance is compared on both sides

EXAMINATION CVS ,CNS AND RS :-

- CVS --- CCF (dysrhythmia)
- CNS --- asterexis
- Renal ---hypocalcaemia

TONSILITIS

Name:

Age:

Sex:

Religion:

Occupation:

Address:

CHIEF COMPLAINTS:

- H/O Fever - _____ days
- H/O Throat pain- _____ days
- H/O Difficulty in swallowing

History of present illness:-

- The patient was apparently normal for past _____ days.
Then he/she develops,

FEVER:-

- Onset : insidious or sudden
- Progressive or not
- Duration :
- Continuous/intermittent/remittent
- Aggravating factor :-
- Relieving factor :-
- Associated with malaise or lethargy

THROAT PAIN :-

- Onset :-
- Progressive or not

- Duration
- Continuous / intermittent :-
- Aggravating factor :-
- Relieving factor :-
- Associated with difficulty in swallowing

DIFFICULTY IN SWALLOWING :-

- Onset :-
- Duration
- Aggravating factor :-
- Relieving factor :-
- History of bilateral ear pain :-
- History of change in voice :-
- History of snoring / mouth breathing :-

NEGATIVE HISTORY :-

- H/O headache
- H/O cough
- H/O nasal discharge
- H/O epistaxis
- H/O ear discharge
- H/O decreased hearing
- H/O ringing sensation

PAST HISTORY :-

- H/O any previous similar complaints
- H/O diabetes mellitus
- H/O tuberculosis
- H/O hypertension
- H/O allergy asthma

- H/O previous surgery
- H/O bleeding diathesis
- H/O blood transfusion

PERSONAL HISTORY :-

- DIET :- Veg or mixed
- Smoking :-
- Alcoholic :-
- Marital status :-
- Degree of consanguinity :-

FAMILY HISTORY :-

- H/O similar complaints in the family
- H/O other systemic disease in the family

Menstrual history :- Regular / irregular

EXAMINATION

VITALS

- TEMPERATURE
- PULSE
- BLOOD PRESSURE
- RESPIRATORY RATE

GENERAL EXAMINATION

- Well oriented to place / time / person
- Well / moderately built and nourished
- Pallor

- Icterus
- Cyanosis
- Clubbing
- Edema
- Lymphadenopathy

ORAL CAVITY :-

- Mouth opening
- Lip and oral commissures
- Tongue
- Floor of mouth
- Buccal mucosa
- Gingivobuccal sulcus
- Dentition
- Hard palate

OROPHARYNX :-

- Halitosis
- Uvula
- Soft palate
- Bilateral anterior pillars
- Tonsils
- Bilateral posterior pillars
- Posterior pharyngeal walls

EAR :-

- Preauricular region
- Pinna
- Posterior auricular region
- Tragal sign

- External auditory canal
- Tympanic membrane
- Middle ear
- Facial nerve
- Eustachian tube

FUNTIONAL EXAMINATION OF EAR :-

- Rinne's test
- Weber's test
- Absolute bone conduction
- Fistula sign
- Spontaneous nystagmus
- Romberg's sign

EXAMINATION OF NECK :-

- Enlargement of lymph node :-(Jugulodigastric)

EXAMINATION CVS ,CNS AND RS

CASE

Name: Dharshan

Age: 8yrs

Sex: Male

Religion: Hindu

Occupation: student

Address: Karur

CHIEF COMPLAINTS :-

- C/O Fever – 4days

- C/O throat pain – 3 days
- C/O painful swallowing – 3 days

HISTORY OF PRESENT ILLNESS :-

The patient was apparently normal before 4 days. Then he develops,

FEVER :-

Onset : sudden

Nature :- Progressive

Duration : 4 days

Intermittent fever

Aggravating factor :- No

Relieving factor :- Medication

THROAT PAIN :-

Onset :- Sudden

Nature :- vague aching pain

Duration – 3 days

Aggravating factor :- Swallowing

Relieving factor :- Medication

DIFFICULTY IN SWALLOWING :-

Onset :- Sudden

Duration - 3 days

Aggravating factor :- Swallowing

History of bilateral ear pain :- No

History of change in voice :- Present

NEGATIVE HISTORY :-

No H/O headache

No H/O cough

No H/O nasal discharge

No H/O epistaxis

No H/O ear discharge

No H/O decreased hearing

No H/O ringing sensation

PAST HISTORY :-

No H/O any previous similar complaints

No H/O diabetes mellitus

No H/O tuberculosis

No H/O hypertension

No H/O allergy asthma

No H/O previous surgery

No H/O bleeding diathesis

No H/O blood transfusion

PERSONAL HISTORY :-

DIET :- Mixed

Non smoker and non alcoholic

FAMILY HISTORY :-

No H/O similar complaints in the family

No H/O other systemic disease in the family

EXAMINATION

VITALS :-

TEMPERATURE – 101 F measured in oral cavity

PULSE – 85 bpm

BLOOD PRESSURE – 130/80

RESPIRATORY RATE – 16 breaths per minute

GENERAL EXAMINATION :-

Well oriented to place, time, person

Well built and nourished

No Pallor

No Icterus

No Cyanosis

No Clubbing

No Edema

No Lymphadenopathy

SYSTEMIC EXAMINATION

ORAL CAVITY :-

Mouth opening - Adequate

Lip and oral commissures - Normal

Tongue – Surface of tongue was coated. No fissures or ulcers. Normal in size

Floor of mouth – Normal. No ulcers and swellings

Buccal mucosa – Normal in colour and normal surface

Gingivobuccal sulcus - Normal

Dentition – Normal

Hard palate – Normal

OROPHARYNX :-

Halitosis was present

Uvula – Red and congested

Soft palate – Red and congested

Bilateral anterior pillars - Congested

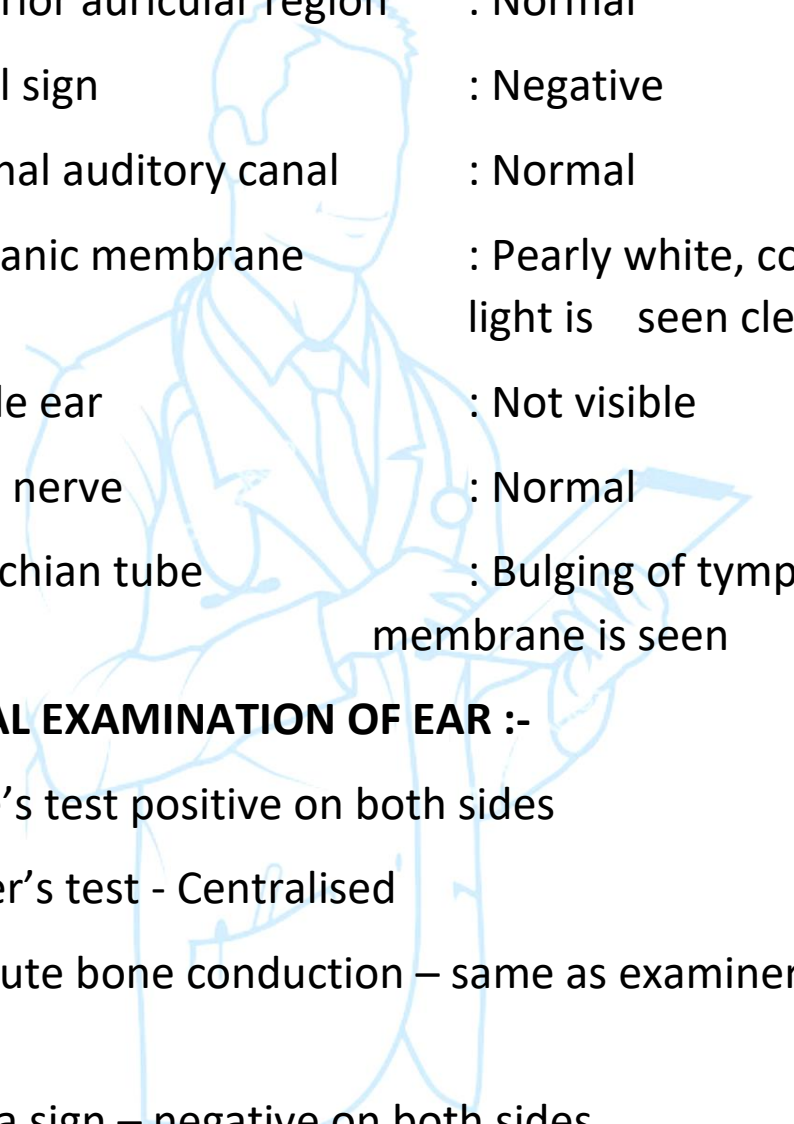
Tonsils – Bilaterally enlarged – grade 3

Bilateral posterior pillars – Not seen

Posterior pharyngeal walls – Congested

EAR :-

Both sides



Preauricular region	: Normal
Pinna	: Normal
Posterior auricular region	: Normal
Tragal sign	: Negative
External auditory canal	: Normal
Tympanic membrane	: Pearly white, cone of light is seen clearly
Middle ear	: Not visible
Facial nerve	: Normal
Eustachian tube	: Bulging of tympanic membrane is seen

FUNCTIONAL EXAMINATION OF EAR :-

Rinne's test positive on both sides

Weber's test - Centralised

Absolute bone conduction – same as examiners on both sides

Fistula sign – negative on both sides

Spontaneous nystagmus - Absent

Romberg's sign - Negative

EXAMINATION OF NECK :-

Enlargement of lymph node :-

Bilateral jugulodigastric lymphadenopathy is present

CVS :-

S1 and S2 heard , No mumurs

RS :-

Bilateral air entry, normal vesicular breath sounds heard

CNS :-

No focal neurological deficit higher mental funtion,
motor funtion

Cranial nerve examination are all normal

The probable diagnosis is bilateral tonsilitis (Grade 3)



TEAM OF DYNAMIC DIAGNOSERS.....

AARTHIKA

AKSHAYA

GEETHANJALI

JAMEEN

KRISHNA KUMAR

MAMTA

MOONITHA

MYTHILI PRIYA

NIRMAL

PRAVITHA

SNEHA

SOWMIYA RAJ

SURJITH

SUVEETHA

TEAM IMPULSE.....

MUKESH KANNA

Thanks!

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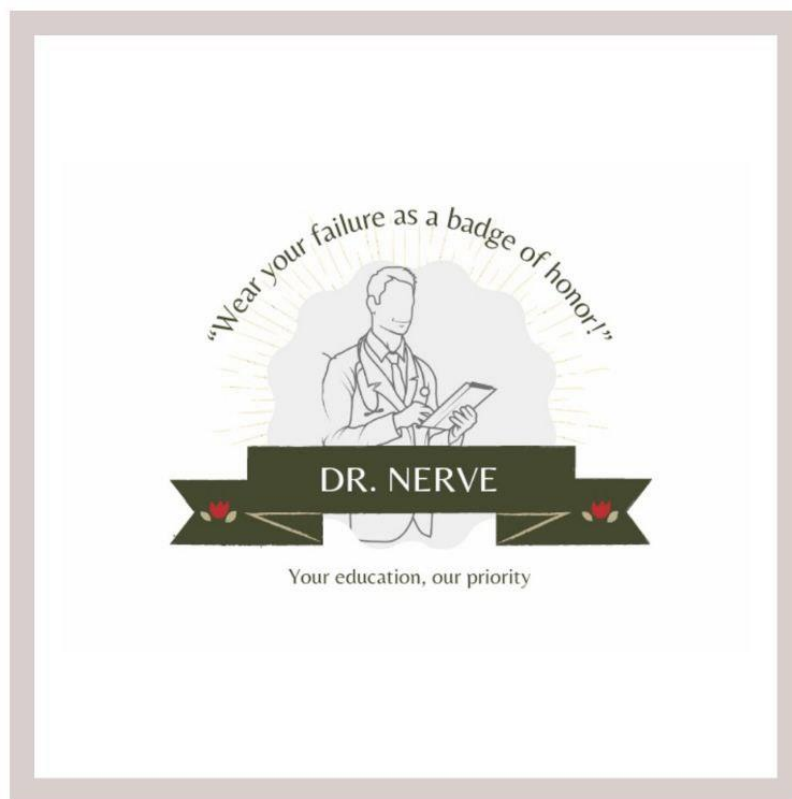


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HOPE IT IS HELPFUL TO U...!!!



K a r u r G M C